Standard Operating Procedure

Manage Product Technical Complaint and Suspected Falsification and Counterfeit Incident

115

Version 14.0

Superseded

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QMS Chapter:	4.9 Product Safety, Product Quality, and Pharmacovigilance

Applicability:

Valid for:	Consumer Health, Pharmaceuticals
Area:	Quality
Shared Relevance:	R&D-QM relevant (Affected)

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Distribution Sheet for Quality Regulations

Division/Business Unit	☒ PH ☒ CH
Product Lifecycle Phase	Product Development ☒ PH Biological Development ☒ PH Chemical & Pharmaceutical Development ☒ CH RD ☒ Manufacturing (Sites) ☒ Commercial Operations ☒ Shared Service Centers
Region/ Country*	☒ Global ☐ EU ☐ US ☐ Specific country: ……
Materials**	☒ APIs (incl. API starting materials, intermediates) ☒ Non-sterile drug products (incl. bulk and combination products) ☒ Sterile drug products (incl. bulk and combination products) ☒ Biotech products ☒ Cell & Gene Therapy products ☒ Medical devices ☒ Cosmetics ☒ Food supplements (for human use)
Relevance for R&D QM	☒ Affected ☐ Interface

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* *Manufactured in or for region / country*
** *Regulation is distributed to each site that handles the materials selected*

Lead Area:	Quality
Replaced Document:	previous version of GSOP 115, Supplement 1636 “RQU Investigation Form for reportable Medical Device incidents and reportable Combination Product malfunction cases”, Supplement 1582 “Reportability Guide for Medical Devices and Combination Products”
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1 Scope and Framework

1.1 Scope

This GSOP defines the process and quality requirements of how to “Manage Product Technical Complaint and Suspected Falsification and Counterfeit Incident”.

It covers the handling, processing and reporting of:

- Product Technical Complaints (PTC) and suspected falsification and counterfeit incidents from the market
- Product Technical Complaints associated with Adverse Events (PTC+AE)
- Product Technical Complaints (PTC) and suspected falsification and counterfeit incidents from internal customers (i.e. other Bayer units to which a Bayer product has been distributed, e.g. for subsequent processing or distribution, where different Qualified Persons are involved)
- Product Technical Complaints (PTC) discovered by Bayer personnel in the exercise of their job, e.g. during installation of a Radiology Medical Device before it is handed over to the customer
- Product Technical Complaints (PTC) and suspected falsification and counterfeit incidents from external customers (i.e. third parties, who receive material from Bayer sites or buy APIs or intermediates)

It applies to:

- all complaints (from internal, i.e. other BAYER sites further processing the products, or external customers) of products manufactured and/or distributed by Bayer AG, its subsidiaries or affiliates.
- all products placed on the market based on contracts for co-distribution, co-marketing and in-licensing as well as contract manufacturing.

Out of scope are:

- Product Technical complaints (PTC) and suspected falsification and counterfeit incidents where BAYER is not the Marketing Authorization Holder (MAH) or the Legal Manufacturer. These cases have to be processed according to Quality Assurance Agreements (QAAs) in place.

1.2 Framework

This GSOP describes one core process of the process group “Manage Quality Issues”.

The overarching framework is provided in the General Process document “Manage Quality Issues” (GSOP 1549).

This GSOP/process has interfaces with:

- GSOP 116 “Manage Product Recall / Field Safety Corrective Action”
- GSOP 117 “Product Quality and Safety Committee (PQSC) - Charter”
- GSOP 257 “Manage GMP/GDP Data Lifecycle”
- GSOP 356 “Manage Corrective and Preventive Action (CAPA)”
- GOI 1034 “Manage reportable medical device complaint”
- GOI 1172 “Perform Periodic Review of Complaints including Quality checks”
- GSOP 1581 “Process EU FMD Alerts”

It is complemented by:

- Supplement 1728 “Root Cause Classification”

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2 Abbreviations

AE	Adverse Event
API	Active Pharmaceutical Ingredient
CMO	Contract Manufacturing Organization
CoQ	Confirmation of Quality Request
CQH	Country Quality Head
CQU	Country Quality Unit
FAR	Field Alert Report
GBS	Global Business Services
HCO	Health Care Organization
HCP	Healthcare professional
IUS	Intrauterine System
LODE	Lack of drug effect
LOE	Lack of effect
PV	Pharmacovigilance
PVCH	Pharmacovigilance Country Head
PQS	Product Quality Safety
PTC	Product Technical Complaint
QA	Quality Assurance
QAA	Quality Assurance Agreement
QVA	Quality Vigilance Agreement
QMS	Quality Management System
RPS	Relevant Production Site
RQU	Responsible Quality Unit
UI	Usability Issue

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3 Definitions

For generic definitions see SUP 200 “Glossary”.

Product Technical Complaint (PTC)	Any report received (written, electronic or verbal communication) about a potential or alleged failure of a product in its quality [including the identity, durability, reliability, safety, efficacy or performance] or a suspected falsification and counterfeit which may or may not represent a potential risk to the patient / customer / user / environment.
Pharmacovigilance (PV) Case	Report which qualifies for capturing in the PV safety database. This includes reports of suspected adverse (drug) reactions (also referred to as individual case safety report (ICSR), adverse events, invalid cases, medical device reports/incidents with potential health impact, serious undesirable effect (SUE) for cosmetic products and certain reports without adverse event referring to: special circumstances (e.g. exposure via parent, lack of drug effect (LODE), medication errors, (intentional) product use issue, off-label use, overdose, abuse, misuse, drug dependency, product use issue,

	pre-existing condition improved (in an unapproved indication) and occupational exposure.
PTC associated with Adverse Event (PTC+AE)	Any report of a PTC or usability issue (UI), which includes information of an adverse event or has been received or initiated in association with a single or cluster of PV cases or constitutes a potential reportable medical device complaint fulfilling any of the following criteria: - a PV case includes a reference to a potential or alleged product technical defect of the product, - a PV case is related to a suspected falsification and counterfeit, - a PV case or cluster of PV case report(s) questions the product quality by the reporter or PV (confirmation of product quality request), - a PV case which refers to a medical device (regardless of incident category), - a PV case which refers to medical device constituent or component of a combination product as suspect product associated to potential or alleged technical or quality defect, - any PTC constituting a potentially (local) reportable medical device complaint even without AE or special circumstances e.g. malfunctions of a device-constituent part of a combination product which might lead to serious injury/death.
Confirmation of Quality Request (CoQ)	Reporter (patient, customer, healthcare professional (HCP), authority, investigator) as well as PV Representatives may raise a “confirmation of product quality request” due to: - reporter or a PV Representative question product quality and/or request confirmation of quality (example: health authority inquiry requesting quality investigation, etc.) - fatal or life-threatening events and other alerting situations which are unexpected for product and patient health condition(s) in which there may be a suspicion of quality issue - unusual cluster or (batch)- signal of similar events which are potentially indicative of quality issue - Medical Device Reporting (21 CFR Part 4 Subpart B) [10]
Incident (EU MDR 2017/745 and IVD 2017/746)	Any malfunction or deterioration in the characteristics or performance of a device made available on the market, including use-error due to ergonomic features, as well as any inadequacy in the information supplied by the manufacturer and any undesirable side effect; and for in vitro diagnostic (IVD) devices, including any harm as a consequence of a medical decision, action taken or not taken on the basis of information or result(s) provided by the device.
Serious Incident (EU MDR 2017/745 and MDCG 2024-4)	Any incident that directly or indirectly led, might have led or might lead to any of the following: a) the death of a patient, user or other person, b) the temporary or permanent serious deterioration of a patient's, user's or other person's state of health, that resulted in any of the following • life-threatening illness or injury, • permanent impairment of a body structure or a body function,

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- hospitalization or prolongation of subject hospitalization,
- medical or surgical intervention to prevent life-threatening illness or injury or permanent impairment to a body structure or a body function,
- chronic disease
- foetal distress, foetal death or a congenital physical or mental impairment or birth defect.

c) a serious public health threat

Serious Public Health Threat (EU MDR 2017/745)

An event which could result in imminent risk of death, serious deterioration in a person's state of health, or serious illness, that may require prompt remedial action, and that may cause significant morbidity or mortality in humans, or that is unusual or unexpected for the given place and time.

Lack of Drug Effect (LODE) / Lack of Effect (LOE)

Failure to produce the expected physiological / pharmacological action of a medicinal product or medical device for human use. The term LODE is synonym to the term Lack of Effect (LOE) if the mode of actions is not a drug substance as in medical devices.

LODE cases may be treated as PTC+AE. The decision is made by the responsible Pharmacovigilance unit.

Usability Issue (UI)

Use error or customer satisfaction issue of a medical device or the medical device constituent of a combination product are each a subgroup of usability issues.

- **Use Error**: Act or omission of an act that has a different result to that intended by the manufacturer or expected by the operator of the medical device or the medical device constituent of a combination product. Use error includes slips, lapses, and mistakes. An unexpected physiological response of the subject does not in itself constitute a use error.
- **Customer Satisfaction issues**: An expression of dissatisfaction by a customer/user, regarding a medical device or the medical device constituent of a combination product because there is no allegation that product fails to meet claims. Customer satisfaction issues include, e.g., handling problems, difficulties to understand or to follow the instructions, etc.

Falsification

Falsification covers:

- Any medicinal **product with a false representation** of its identity, including its packaging and labelling, its name or its composition (other than any unintentional quality defect) as regards any of its ingredients including excipients and the strength of those ingredients
- **Manipulations** of genuine product for instance but not limited to (1) changing variable data, (2) removing/exchanging content, (3) returned and resold used products
- **Disappeared/Diverted Product:** Genuine product rerouted out of its intended legal supply chain into another supply chain by unauthorized actions, irrespective of whether the second supply chain is legal or not
- **Disappeared/Theft/Stolen Product:** Genuine product taken without authorization from anywhere in the upstream or downstream supply chain (warehouse, transport means, whilst

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within transit), be it by use of force, threat of force (e.g., robbery), misappropriation (e.g., by personnel) or deception (e.g., fraud)

Remarks

Disappeared products in the sense of “a **lost product**”, meaning (in contrary to diverted or stolen products - see definition above) the accidental misplacing or disappearance of a product with the result that its whereabout is unknown at a specific time and, condition that there is not intent involved in the displacing or disappearance, are a separate category.

These incidents are processed as complaints (quantity issues).

Genuine product which is initially in doubt to be genuine and therefore reported as suspected falsification / counterfeit are processed as regular PTCs.

Counterfeit

Counterfeit covers:

- **Trademark Violation:** Product either using without authorization any or a combination of the following: tradename, trademark, icon, rights owned and protected by Bayer according to applicable legislation.
- **Fraudulent products**: “Products imitating a “generic” version of a genuine (Bayer) product by using any or a combination of the following (1) visual aspects of the original, (2) naming very close to the original or (3) claiming that the same API is present and independent of its registration status and true composition by excluding generic products with known legal registration status which belong to an identifiable MAH.”

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4 Process Information

4.1 Purpose and General Guidance

Managing Product Technical Complaints (PTCs) including setting effective CAPAs are a mandatory core element of any QMS. It is BAYER´s high priority obligation to monitor the safety and quality of its products and processes for the benefit and safety of patients, customers, users, and environment.

Managing PTCs comprises the collection, documentation, evaluation, reporting quality related information, plus respective escalation of potential critical issues to management functions and Health Authorities. This also includes the management of suspected falsification and counterfeit incidents.

4.1.1 Complaint Classification

Product Technical Complaints and Suspected falsification and counterfeit Incidents are classified initially and finally after their evaluation. The classification is based on the company's assessment of the nature of the case and the potential risk to patient / customer / user / environmental safety according to the following definitions:

Risk Classification [8]	**Initial**	**Final**
High	Complaints regarding **potential** defects which are potentially life-threatening or could cause a serious risk to health.	Complaints regarding defects which are potentially life-threatening or could cause a serious risk to health.
Medium	Complaints regarding **potential** defects which could cause illness or mistreatment with potentially non-serious medical consequences but are not classified as high	Complaints regarding defects which could cause illness or mistreatment with potentially non-serious medical consequences but are not classified as high
Low	Complaints regarding **potential** defects which **may not** pose a significant hazard to health.	Complaints regarding defects or alleged defects which **may not** pose a significant hazard to health.
Non-Justified	Not to be used	Non-defective product or the investigation did not confirm the alleged defect

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For Radiology medical devices product non-malfunctions, final risk classification and severity category are based on the Final Patient Outcome that aligns with Radiology the Risk Management Files.

Examples for each risk classification are listed in Annex 1: Examples for Risk Classification of PTCs.

4.1.2 Timelines

Complaint timelines are based on the depth of complaint assessment / investigation required. The "Total max timeline" (see table) is defined by the time required to complete all activities. After the initial notification of the GBS/CQU (date becoming aware/ awareness date) the duration of the complaint process adheres to the following time frame [in calendar days]:

Complaint process type	Complaint Initiation by GBS/CQU	Investigation[2]	Approval / Closure[2]	Total max. timeline[1]
Standard	5 (immediate escalation within 24 hours as required according to 5.1.2)[5]	20	5	30
Non-Standard[3]		50	5	60
Local Closure[4]	30	-	-	30

[1] For due date extensions, refer to chapter 5.4.1

[2] plus remaining time from previous step(s)

[3] Non-standard means

- a third party / CMO / external lab (service provider) is involved
- sample was requested but not received during data entry step within the required timeline.
- sample shipment is difficult
- an extension of investigation or testing is required (e.g. analysis at external laboratory, quality safety evaluation)

[4] see section 5.1.1.1

[5] immediate escalation necessary, e.g., for serious public health threats, potentially life-threatening cases, cases potentially requiring a FAR

The awareness date in the complaint database reflects the day that BAYER Quality function becomes aware of the complaint.

If an amendment of an already existing complaint case is necessary, the awareness date remains the date when entering BAYER Quality function originally became aware of.

If new relevant information is received after the closure of the complaint case, a follow-up case is opened with a new awareness date and clock starts again.

4.1.3 Sample Handling

Any sample can serve as evidence in a legal case. Therefore, the handling has to be done in a controlled manner. The loss of such sample can lead to potential liability or criminal sanctions.

If samples received in an evidence bag, they remain associated with the original sample bag, and the integrity of the bag and its content are maintained. The chain of custody is always documented (in some cases directly on the evidence bag) so that it can be provided if and as requested by the authorities.

Any sample returned with a chain of custody is handled in a manner that maintains the integrity of the chain of custody established by the reporter. This is usually a government agency that will provide instructions to follow. Return sample shipments must be traceable through the logistics service employed.

If all necessary information (e.g., all variable data, any serialization code with its human readable information and texts printed on the product, etc.) can be provided by suitable electronic evidence (e.g., photo, video) or if shipment creates additional risks (e.g., safety hazard, biohazard), shipment of samples should be substituted by photo or video. The complainant is asked to keep the sample, if

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available, for at least 3 weeks. The RQU notifies GBS/CQU regarding sample requirement for investigation.

For IUSs BP 1429 “IUS Technical Complaints Guidance document” needs to be followed.

Complaint samples are retained for at least 3 months after completion of the investigation by the RQU, an assigned RPS or the CQU if investigation is not needed at the site. Samples of suspected falsification and counterfeit drug products and parts of them such as broken ampoules, emptied syringes, partially used blisters are also stored for a minimum of 3 months after completion of the investigation if not advised otherwise by e.g., legal counsel for instance because they are needed as evidence in legal proceedings. Upon request of legal counsel, the samples are sent to the designated internal function or any other assigned external lawyer, investigator, or person. There the samples are stored as long as deemed necessary by legal counsel.

4.1.4 Final classification of Complaint cases

Terms	Definition
Confirmed quality defect	The initial complaint (alleged quality defect, issue, error) could be confirmed, and the root cause of the confirmed quality defect is within Bayer responsibility.
Unconfirmed quality defect	The complaint (alleged quality defect, issue) is unconfirmed, **or** root cause of the confirmed issue is not within Bayer responsibility.
Unconfirmed quality defect, but plausible	The alleged quality defect / issue can neither be confirmed nor ruled out by the RQU to be within Bayer responsibility but is plausible from a complaint description point of view.

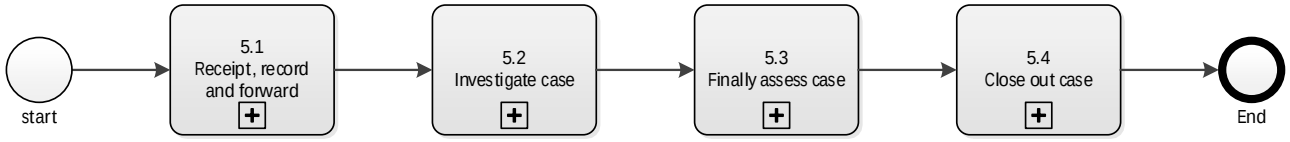
4.1.5 Product and case specific requirements:

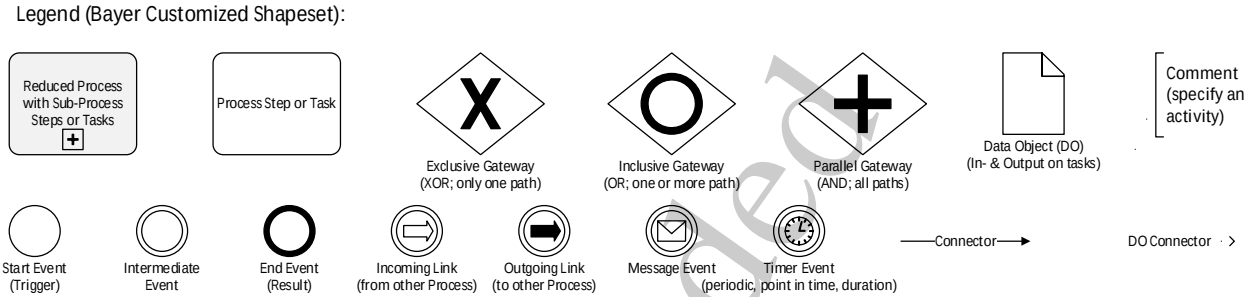
- For combination products registered as medicinal drugs the rules of complaint handling for medicinal products have to be followed. Product specific requirement needs to be considered.
- Any product specific procedure for handling of technical complaints has to be approved by the divisional QA and documented.
- The complaint process for specific low risk Consumer Health products (i.e. cosmetic and food supplements) follows risk-based approach. A single case evaluation is done at RQU level to determine the need of investigation based on pre-defined criteria (see below). Investigation is mandatory and whenever considered relevant, if:
    - Initial risk classification “high” or “medium"
    - Complaint involves Health Authority
    - Suspected falsification and counterfeit, if sufficient information to perform an investigation is available (i.e., sample and/or pictures)
    - Confirmation of Quality Request
    - RQU detects specific trend (e.g., for a batch, specific issue, country, error code)
    - Signal/Trend detected during periodic review in accordance with GOI 1172 “Perform Periodic Review of Complaints including Quality checks”
    - Investigation is mandatory if required by local regulation
- PTCs from internal customers (i.e. other Bayer sites, where the products are further processed) or external customers (i.e. third parties, who receive material from Bayer sites or buy APIs or intermediates) follow the same process as PTCs from the market. The tasks of GBS/CQU are resumed by the detecting site.
    - Case entry is conducted by the detecting site.

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- Investigation, initial and final risk classification (including the assignment of initial and final error codes), and final assessment are conducted by the RQU of the original manufacturing site.
- Close out and decision for further processing of internal complaints are performed by the detecting site

4.2 Process Flow





4.3 Reference to Process Description

Detailed description of the process can be found in Chapter 5 and is clustered as follows:

- Receipt, record and forward case (→ 5.1)
- Investigate case (→ 5.2)
- Finally assess case (→ 5.3)
- Close out case (→ 5.4)

4.4 Assigned Roles and Responsibilities

Global Business Services (GBS), legal entity of BAYER AG

- performs the initial assessment of complaints after receipt and considering the information provided by the PVCH or PV PQS in case of an AE and/or a medical device (incl. medical device constituent of combination product) being involved.
- informs CQU immediately about all complaints and PTCs where local authorities are involved and about all potential falsification and counterfeits
- supports the process for providing the complaint sample, for reimbursement management and responses to the complainant
- ensures translation of non-hosted languages
- informs the PVCH about all PTCs with a potential associated AE or LODE or potentially reportable medical device complaints including the unique complaint database case number and all relevant follow-up information (e.g., batch number).

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- enters the PTC / PTC+AE, suspected falsification and counterfeit incidents and usability issues according to defined timelines into the complaint database in English language.
- checks completeness and accuracy of information (including the photo or video, if available from the complainant) before forwarding the cases
- Updates the complaint database regarding sample flow/status and notifying the CQU when sample retrieval is required
- Provides instructions to the complainant on further sample processing, e.g., instruction to send the sample directly to the RQU or CQU
- Initiates first contact with the complainant within first 5 days of receipt of the complaint information to obtain follow-up information (e.g. batch number, event details, etc.) and complaint sample, as needed, documenting these efforts including the number of attempts requesting sample.
- responds to the complainant, for PTC+AEs in liaison with the PVCH and documents the response in the complaint database or other cross-referenced system in cooperation with the CQU.
- notifies the legal manufacturer of a device and where necessary, the manufacturer's authorized representative and the importer, if applicable.
- Finally closes the complaint case

The responsibilities of the GBS can be also performed by the Country Quality Unit (CQU), if not transferred.

Country Quality Head (CQH) / Country Quality Unit (CQU) of the legal entity which introduces product into market (e.g. Marketing Authorization Holder, Distribution license holder) or in clinical trials

- ensures the appropriate handling of PTCs, and suspected falsification and counterfeit incidents, reported in the respective country including the timely execution of the related activities, particularly for the entry of the cases into the complaint database, forwarding them and for responding to the complainant.
- responds to the complainant, for PTC+AEs in liaison with the PVCH and documents the response in the complaint database or other cross-referenced system in corporation with the GBSs.
- fulfills all obligations, e.g., PTC related reporting requirements defined by legislations or regulations.
- Oversees and monitors the whole PTC process for products on market of their legal entity in case of delegation to GBS, e.g., by controlling appropriate Key Performance Indicators according to GSOP 1372 “Perform Country Quality Management Review”

Pharmacovigilance Country Head (PVCH)/ Product Quality Safety (PV PQS)

- informs the GBS/CQU about PV cases which qualify for a PTC+AE.
- provides to the GBS/CQU all available PTC relevant case information, including reason for PTC, case seriousness, incident

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category for medical device related cases and all relevant follow-up information.

- For PVCH only: receives from GBS/CQU and captures any PTC constituting a medical device complaint with potential health impact even without AE or special circumstances

Divisional Quality Assurance (QA)

- ensures that the tasks pertaining to the complaint handling process and suspected falsification and counterfeit incidents are fulfilled and that the described responsibilities are assigned to functions within the organization.
- ensures that for each marketed product a Responsible Quality Unit (a quality unit that released the product onto the market, or a central service function, e.g., for a particular region or for products produced and released by contract manufacturers.) is available.

Gatekeeper (GK)

See GSOP 117 “Product Quality and Safety Committee (PQSC) - Charter”.

- demands a higher or a lower risk classification than initially assigned, if necessary, based on an appropriate justification which has to be documented.

Responsible Quality Unit (RQU)

- assesses initial PTC reports for correctness and completeness of information and assigns initial complaint classification.
- ensures that adequate technical and risk-based investigations (including root cause investigation and potential CAPAs) are performed in a timely manner and documented appropriately in English language and in the complaint database.
- ensures that complaint information is appropriately forwarded to relevant functions.
- notifies the legal manufacturer of a device or the manufacturer's authorized representative and the importer, if applicable according to the established QAA/QVA.
- ensures definition and implementation of necessary corrections, corrective and preventive actions
- issues a final assessment and provides GBS/CQU with a final statement.

For complaints received from internal customers (i.e. other Bayer sites, where the products are further processed) or external customers (i.e. third parties, who receive material from Bayer sites or buy APIs or intermediates) the RQU:

- initiates a complaint in case (including complaint classification) that a defect of a product received from another Bayer manufacturing site is detected
- coordinates the complaint processing after receipt of a complaint from a Bayer site or an external customer, especially if other Bayer sites or contract manufacturers are involved in the manufacturing of the product
- ensures response to internal or external customers

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For Medical Devices the Legal Manufacturer is responsible for the investigation and assessment including escalation of the complaint case in their respective complaint database.

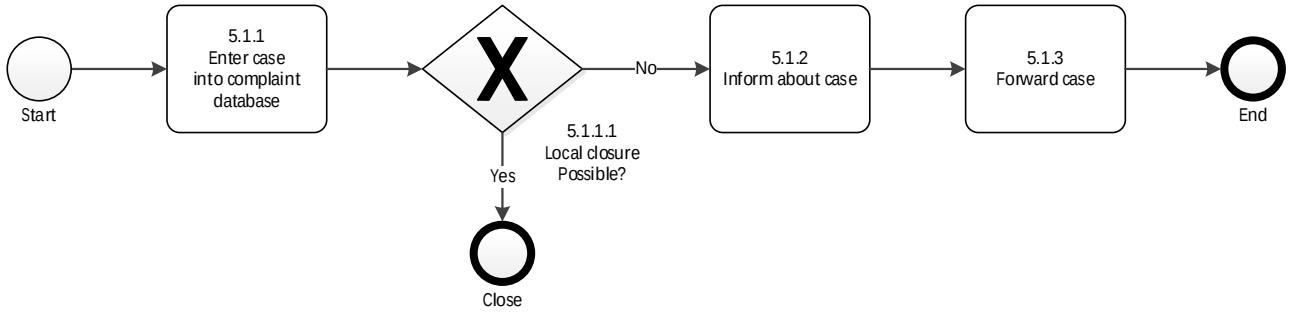
Relevant Production Site (RPS)

- decides on the adequate level of investigations and analysis.
- carries out the investigations (including root cause investigation) in a timely manner and documents the results obtained.
- retains the complaint samples including samples of suspected falsification and counterfeit products.
- implements necessary corrective and preventive actions.

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5 Detailed Process Description

5.1 Receipt, record and forward case



Description	Assigned Roles & Document Reference
5.1.1 Enter case into complaint database – File each case (even those for already expired products) including suspected falsification and counterfeit incidents, usability issues for medical devices and combination products as complaint in the complaint database within the required timeline (see 4.1.2). – Screen, if the case represents a serious public health threat, is potentially life-threatening or requires a FAR and escalate immediately according to 5.1.2. – Ensure completeness and accuracy of the information and add all available documentation (also documents in local language). – Use Annex 4: Pick list for Reporting Suspected Falsification and Counterfeit Incidents to facilitate and harmonize reporting on suspected falsification and counterfeit Incidents. – Perform up to 3 documented attempts in the complaint database to obtain missing or insufficient relevant information such as batch number, applied serial numbers for suspected falsification and counterfeits and the complaint sample required for the complaint investigation unless justified why less or even no attempt was made, e.g. if a sample is declared not to be available. – Process medical device and combination product complaint with potential health impact (Serious Public Health Threats, Incidents) as PTC+AE to ensure that a device report can be submitted to Competent Authorities and the responsible Notified Body via the global safety database as needed. – Process all Radiology Medical Devices complaint cases, i.e. PTC, PTC with AE and AE cases in the complaint database only and not in the safety database. – Enter usability issues into the complaint database, even those with no report of a suspected quality defect. – Process use errors as PTC+AEs if they are associated to a PV case, i.e. reported AE or special circumstances, e.g. off-label use, misuse, medication error. – Process LODE cases as PTC+AE if PV raises a confirmation of product quality request – Determine if a technical investigation is necessary. If not, provide justification. – Document and maintain all information related to the receipt of complaint cases in the complaint database according to GMP documentation rules according to GSOP 257 “Manage GMP/GDP Data Lifecycle”.	GBS/CQU GSOP 257

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Description	Assigned Roles & Document Reference
– Send an acknowledgement letter, if required, to the complainant using a template defined in local SOP.	
Remarks:	
– First attempt to retrieve the samples / pictures / batch number associated with the complaint is completed and documented within 5 days of complaint information receipt. After first attempt is completed, complaint is forwarded for assessment, only if • Sample has been shipped by the customer; or • sample has been indicated to be available, but first attempt was unsuccessful in retrieving the sample, but pictures were provided; or • Batch number has been provided by the complainant / reporter and included in the complaint database	
– If the complaint database is not available, cases are documented by using Annex 2: Report Form for PTC or suspected Falsification and Counterfeit Drugs. The data are entered as soon as the database is available again.	
– Personal Data are only entered with a distinct purpose, e.g., retrieval of sample or providing a response to the complainant and if permitted by local law.	GOI 504
– For countries/sites performing repackaging activities or visual inspections, internal complaints only have to be forwarded to the manufacturing sites if predefined action levels or AQLs (according to GOI 504 “Perform AQL-Sampling and Defect Evaluation”) are exceeded: • Product specific defects are defined by the country organization or proposed by the site in line with Supplements 505-507 • All major and critical defects identified during repacking activities or visual inspections are entered into the complaint database • Minor and cosmetic defects are only entered into the complaint database, if predefined action levels based on historic data are exceeded (see also GOI 582 “Requirements for Visual Inspection of Parenteral Final Products”). Defects below action levels can be forwarded per email monthly to the respective manufacturing site for information.	SUP 505-507 GOI 582
– Bundling of cases is only acceptable for • Suspected falsification and counterfeit incidents, e.g., due to serialization alerts with initial classification low, if they are related to one single product, one single batch and one single country (multi-country pack located in "EU" is considered as one country) received on the same day (see also GSOP 1581 “Process EU FMD Alerts”). • Social media triggered complaints or complaints from Marketing studies, e.g., related to the same defect, received in a short time frame, by one single country, assessed as unconfirmed quality defect and in alignment with divisional quality.	GSOP 1581
– Such bundled cases are handled in the following way: • Cases are collected in the complaint database under one case for one product with a short description by the GBS/CQUs for social media triggered complaint or Alert Operators for Suspected Falsification and Counterfeit Incidents. To conduct this the alert operator creates an entry in the Complaint Management System daily and forwards the case to the CQU/ GBS. This allows appropriate trending and signal detection.	

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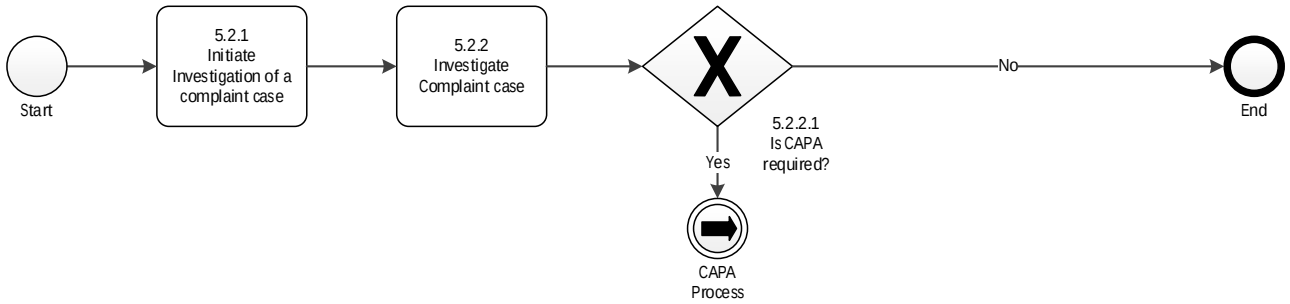
Description	Assigned Roles & Document Reference
5.1.1.1 Local Closure possible? – Only for cases associated with a. Usability Issues or b. Documentation or c. PTC, where • legal manufacturer was informed and • no complaint sample is available and • no photo of the sample can be sent and • no batch/serial data of the drug product or medical device have been reported after three documented attempts and • no AE was reported include the rationale for local closure provided that three documented attempts were made to obtain the sample and the necessary information or sample is already declared not available. **Remarks:** – For complaints closed locally, classify complaint case using risk classification according to its criticality (see 4.1.1)	GBS/CQU
5.1.2 Inform about case – Inform the respective PVCH immediately and provide full case information, if a PTC or a suspected falsification and counterfeit incident includes information of a (potential) PV case / Adverse Event. – Escalate immediately to divisional Gatekeeper if the case is considered to be potentially life-threatening or could cause a serious risk to health. – Inform the CQU immediately if a PV case involves a PTC or if a confirmation of quality request (CoQ) is raised by an external (e.g. health authority) or internal (e.g. monitoring observation) function and provides all available PTC relevant case information or suspected falsification and counterfeit is reported. – Forward PTC-related complaint information including suspected falsification and counterfeit incidents to local authorities, if required by local law. – Assess the reportability of a medical device complaint and the reporting of medical device cases to authorities, Notified Bodies, 3rd party legal manufacturers itself out of the global safety database (see GOI 1034 “Manage reportable medical device complaint”) – Assess the reportability of a constituent Part of Combination product and the reporting of cases to FDA out of the global safety database (GOI 1034) **Remarks:** – Radiology Medical Device MDR reporting is done independent to the complaint database. – In case of suspicion of falsification and counterfeit incidents falsification and counterfeit notification groups are informed – Local timelines have to be considered, e.g., for registered pharmaceutical products distributed in the US, a Field Alert Report (FAR) is submitted within 3 working days for any deterioration (e.g. trend of complaints) in any distributed product and for potentially critical complaints where the affected batch(es) is/are distributed to more than one country, even if the investigations have not been finalized.	GBS/CQU GBS CQU PVCH / PV PQS GOI 1034 PV

Superseded

Description	Assigned Roles & Document Reference
– For approved biological products distributed in the US, a Biological Product Deviation Report is submitted per CFR 600.14 not to exceed 45-calendar days from the date a reportable event has occurred. – For EMA centrally authorized medicine, including suspected defects, an EMA DPR ("Defective Product Report" Process to Report a quality defect of a centrally authorized medicine, including suspected defects, to EMA) is submitted.	
5.1.3 Forward case – Forward the complaint to next step, if pre-requisites for local closure are not met as defined in chapter 5.1.1.1. – Contact the RQU for further instructions in case of doubts whether the sample has to be shipped or not. – Forward all samples and/or pictures capturing relevant details without undue delay and according to defined timelines for investigation to the Responsible Quality Unit – Organize the shipment of the complaint samples including package and labelling in accordance with pertinent safety and regulatory requirements – Forward any relevant follow up information on complaints to the RQU and to the PVCH in case of a PTC+AE.	GBS/CQU

Superseded

5.2 Investigate case



Description	Assigned Roles & Document Reference
5.2.1 Initiate investigation of a complaint case – Assess initial PTC case documentation for correctness, completeness of information or send it back to the GBS/CQU for correction, if necessary – Assign initial Risk Classification according to its criticality (see chapter 4.1.1) – Assign or update initial error codes – Notify GBS/CQU regarding sample requirement for investigation (sample needed / not needed) – Initiate and coordinate the investigation and evaluation of the complaint, if necessary, with support of other production sites involved in the manufacturing, packaging or release of the product, including contract manufacturers where applicable. – Escalate case immediately (if not already done before) to the divisional Gatekeeper, if case is potentially life-threatening or could cause a serious risk to health. – Prioritize the investigation of cases classified as “medium” and “high” striving to complete investigation as soon as possible – Notify the legal manufacturer, if needed **Remarks:** – GBS/CQU is informed via a system report, if initial risk classification is “high”. – No investigation is required for usability issues for product incidents that have been determined to be the result of use errors and/or customer satisfaction issues and are known, unconfirmed issues. However, if there is an associated PV case that is directly related to the Usability Issue, the case has to be entered into the complaint database as a single case (unless a product specific agreement for an alternative process has been agreed). For all new medical devices and combination products incoming cases are reviewed for a period of time after launch (timelines are defined by responsible Divisional QA) to identify specific Usability Issue categories. These reviews are formally documented to justify inclusion as a Usability Issue. – When a specific complaint reason is well understood (i.e., the root cause including potential CAPAs and associated risks could be identified during the investigation of a previous complaint on the same batch), a reference to an investigation already performed can be used for a given batch of product and a specific complaint reason. An investigation report is prepared by the RQU for the first complaint received for that batch and	RQU

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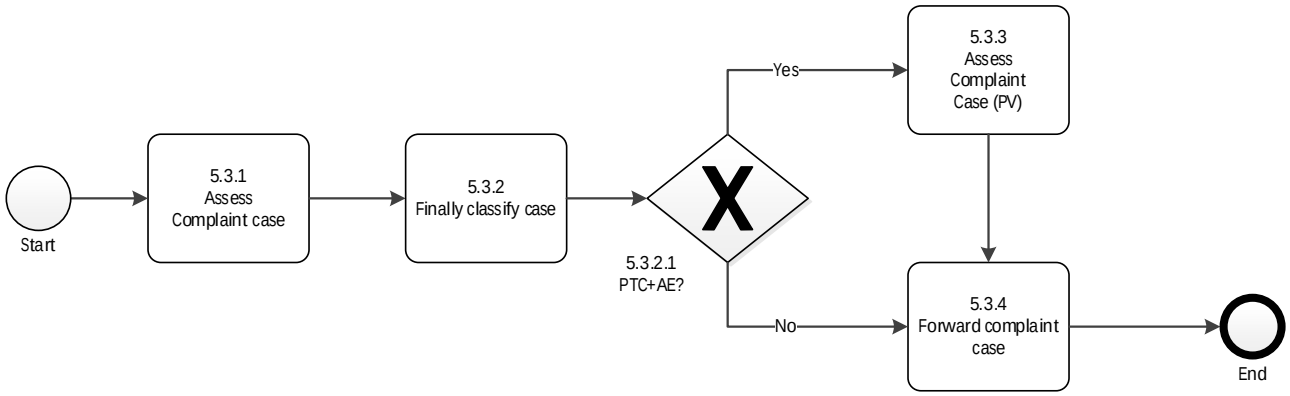
Description	Assigned Roles & Document Reference
kept in the complaint database under the first complaint case initiated. The investigation report of the firstly reported case is utilized for additional complaint cases handled under separate case numbers for this particular batch and complaint reason. Prerequisites to investigate as described above: 1. The complaint type is an initial risk classification of low. 2. The investigation is for a specific PTC type (e.g. adhesion) and an individual batch number. 3. The investigation of the first case of a batch is not older than 6 months unless a plausible justification is provided why no new investigation has to be requested or a pre-defined threshold is not exceeded.	
5.2.2 Investigate complaint case – Perform a root cause analysis – Review executed batch record, testing and/or release documentation, as appropriate, which covers but is not limited to: • in-process control results (IPCs) • calculation of intermediate and final yield(s) • out of specification (OOS) results • (other) events or deviations • cleaning processes, line clearance, monitoring results of routine cleaning process • environmental monitoring results • results of visual inspection • any associated relevant documentation related to the production of the specific batch such as cleaning records, calibration records, maintenance of equipment. • In case of suspected falsification and counterfeit: safety features and applied serial numbers. For the EU market their status and event information is retrieved from the National Medicine Verifications Systems (NMVS) and also made available through the European Medicines Verification System (EMVS) and/or the internal status is retrieved via the internal global serialization database (ATTP=advanced track and Trace for pharmaceuticals). For other countries the status information is retrieved by a portal, web interface, webpage or application or similar, depending on which tool is made available. – Investigate and/or assess raw material, starting material and packaging material, if necessary – Investigate complaint sample by visual inspection and chemical, physical or microbiological analysis according to Annex 5: Decision tree for handling complaint samples – Investigate retain samples by visual inspection of reference and retention sample (a justification is provided if visual inspection is not performed) and chemical, physical or microbiological analysis of reference samples as appropriate – For suspected falsification and counterfeit cases follow instructions in form SCEF (Falsification and Suspected Counterfeit Evaluation form) to be mandatory filled for such cases if pictures and or samples are available (see Annex 3: Falsification and Suspected Counterfeit Evaluation Form (SCEF)).	RPS, QC, RQU

Superseded

Description	Assigned Roles & Document Reference
– Determine a root cause according to Supplement 1728 “Root Cause Classification” – Assess the potential impact on other batches or products – Review previous quality defect reports (with the same batch number or for the same product) for any indication of recurring problems requiring attention – Assess for Medical Devices or Combination Products whether a usability error can be confirmed – Maintain, either by referring an ID of an established system or by adding the documents to the case, all documents generated with regard to the complaint investigation, sample analysis and assessments **Remarks:** – If a suspected falsification and counterfeit case is investigated by a third party an alternative process than the SCEF may be followed preconditioned that sufficient details to confirm or exclude a falsification and counterfeit are included in the investigation report. – Testing is designed to confirm the quality of the product/batch by data.	Supplement 1728
5.2.2.1 Is CAPA required? – If yes, determine CAPAs according to GSOP 356 “Manage Corrective and Preventive Action (CAPA)” – If no, justify why CAPA is not required in case of confirmed defects	RQU GSOP 356

Superseded

5.3 Finally assess case



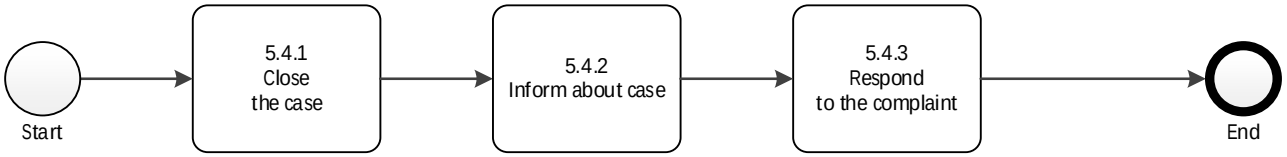
Description	Assigned Roles & Document Reference
5.3.1 Assess complaint case (Q) - Ensure that adequate investigations are performed in a timely manner and that it is documented appropriately in the complaint database in English language. - Ensure the completeness of investigation and request additional investigation, if needed. - Ensure that a root cause or potential root cause has been identified. – Assess the complaint regarding the alleged quality issue, performance issue, usability issue or falsification and counterfeit incident including confirmation or non-confirmation of a product quality defect in the complaint database according to the definitions in chapter 0. – Assess the potential impact and risk posed by the quality defect based on its severity and extent. – Review for all device cases where BAYER is the Legal Manufacturer whether the event and its frequency of occurrence is anticipated or not according to the device risk management file and relevant FMEAs. – Assign the final defect type and root cause. – Provide and document rationale in case use error has occurred. – Review and approve investigation report, if applicable. – For Pharma: For Medical devices and Combination Products (device constituent part affected) enter the respective IMDRF codes for potentially reportable cases into the complaint database – For Consumer Health: For potentially reportable cases IMDRF codes and Device Problem codes are evaluated jointly between Quality and Pharmacovigilance functions. – Request extension to complaint timeline, if the complaint cannot be completed within the scheduled due date due to the nature of the test, e.g. for microbiological tests, or if third parties are involved (contract manufacturers). **Remarks:** – For Radiology a “confirmed quality defect” is selected when a CAPA has been initiated. – For Radiology FDA codes are entered on electronic submissions to the FDA and are not subsumed in the complaint database.	RQU

Superseded

Description	Assigned Roles & Document Reference
5.3.2 Finally classify case – Assign the final risk classification according to chapter 4.1.1. – Escalate case to the divisional Gatekeeper, if at any time during the investigation the results indicate a quality issue with classification “high” via the complaint database. **Remarks:** – Inform GBS/CQU via a system report, if final risk classification is “high”.	RQU
5.3.2.1 PTC+AE? – If yes, assess if case must be forwarded for Quality Safety Evaluation after conclusion of the technical evaluation to PV. Provide sufficient time before closure, i.e. at minimum, 1 working day for final risk classification “High” cases, 3 working days for final risk classification “Medium” cases and 5 working days for final risk classification “Low” cases. – If no, proceed with 5.3.4 **Remarks:** – For cases that are considered as PTC+AE, those must be forwarded to PV for Quality safety Evaluation, if • Conclusion is confirmed quality defect or unconfirmed quality defect but plausible • Confirmation of Quality Request is raised • Suspected falsification and counterfeit is reported • Conclusion is unconfirmed quality defect and medical device or combination product (device part is affected) is involved • Unconfirmed quality defect (with new significant information to be added/updated, i.e., PTCs or batch number)	RQU
5.3.3 Assess complaint case (PV) – Complete quality safety evaluation regarding the potential correlation between a confirmed or plausible technical defect or use error and the reported Adverse Event.	PV PQS
5.3.4 Forward complaint case – Forward the complaint for final verification and closure to the GBS/CQU according to the defined timelines see chapter 4.1.1. **Remarks:** – In case of confirmed falsification and counterfeit incidents respective functions within the organization are informed.	RQU

Superseded

5.4 Close out case



Description	Assigned Roles & Document Reference
5.4.1 Close case – Finally verify investigation report regarding completeness and comprehensiveness after becoming aware of the case according to timelines in chapter 4.1.2 – Ensure completeness of all case related documents either attached to the case or by reference to further record keeping storage places. – Close the complaint case in the complaint database **Remarks:** – Initial Receipt Date is in all cases (initial and follow-up complaints) the date BAYER Quality function first became aware of the original PTC. – Due date extension of a complaint is allowed, with interim report and justification, if complaints cannot be closed within defined timelines. However, all complaints have to be finally closed after 90 days the latest based on the available information at that time. – If all complaint related activities cannot be completed within 90 days (e.g., where the investigation includes third party and/or external service provider such as external lab and/or expanded over multi-sites), complaints remain open beyond 90 days and are considered overdue. GBS/CQU is notified about the current status. – If new information is received after closure of the complaint case that changes the outcome of the investigation a follow-up complaint must be initiated (i.e., sample received which was not available initially, patient outcome has changed) and the clock starts again (with new awareness date) according to the timelines in chapter 4.1.2. – In case a Field Alert Report (FAR) is submitted, the divisional QA provides the information for referencing the FAR in the investigation report as well in the complaint database. – Complaints are amended only for corrections or to add pertinent information related to the complaint after its closure, e.g., email from a health authority closing an incident, revised response letters. Reopening must be requested to and approved by QA. – Deletion of personal data in closed complaint cases is handled by the complaint process change control, if requested by the complainant / reporter.	GBS/CQU
5.4.2 Inform about case – Ensure that in coordination with Regulatory Affairs information of complaint case is forwarded to authorities according to local requirements and timelines and in alignment with PQSC. – Inform all immediate trading partners as required by the *"Guidance for Industry on Drug Supply Chain Security Act Implementation: Identification*	CQU

Superseded

Description	Assigned Roles & Document Reference
of Suspect Product and Notification" not later than 24 hours after making the determination	
5.4.3 Respond to the complainant – If requested by authorities or individual complainant (e.g., HCOs, HCPs) or required by local regulations: • Ensure that a written response is provided in a clear comprehensible form including a company evaluation and a statement concerning the root cause of the defect, if available. • Consult the PVCH to ensure a coordinated response is provided for PTC+AE. – Document and maintain all correspondence regarding the response **Remarks:** – The response does not consist of copies out of the complaint database.	GBS/CQU

Superseded

6 Internal References

Procedure PHG-PUB-000010	Anti-Counterfeiting
Corporate Policy No. 2095	Product Safety and Quality: Reporting Obligations of Employees
GSOP 116	Manage Product Recall / Field Safety Corrective Action
GSOP 117	Product Quality and Safety Committee (PQSC) - Charter
GSOP 208	Perform Site Quality Management Review
GSOP 257	Manage GMP/GDP Data Lifecycle
GSOP 356	Manage Corrective and Preventive Action (CAPA)
GOI 504	Perform AQL-Sampling and Defect Evaluation
SUP 505	Classification of Defects for Bulk and Final Product Solids
SUP 506	Acceptance Criteria for Liquids and lyophilized products
SUP 507	Acceptance Criteria for Semisolids
GOI 582	Requirements for Visual Inspection of Parenteral Final Products
GOI 1034	Manage reportable medical device complaint
GOI 1172	Perform periodic review of complaints including Quality checks
GSOP 1372	Perform Country Quality Management Review
GSOP 1581	Process EU FMD Alerts
SUP 1728	Root Cause Classification
BP 1429	IUS Technical Complaints Guidance document

7 External References

[1] 21 CFR 820.3 – Quality System Regulation - Definitions

[2] 21 CFR 600.14 - Reporting of biological product deviations by licensed manufacturers

[3] REGULATION (EU) 2017/745 OF THE EUROPEAN PARLIAMENT AND OF THE COUNCIL of 5 April 2017 on medical devices

[4] EU GMP Guide, Part 1, chapter 8 “Complaints and Product Recall”, 13-08-2014

[5] Guidance for Industry on Drug Supply Chain Security Act Implementation: Identification of Suspect Product and Notification, June 2021

[6] Recommendation CM/Rex(2024)3 of the Committee of Ministers to member States on reporting of disappearances of medicinal products for human and veterinary use from legal supply chain

[7] DIRECTIVE 2011/62/EU OF THE EUROPEAN PARLIAMENT AND OF THE COUNCIL of 8 June 2011 amending Directive 2001/83/EC on the Community code relating to medicinal products for human use, as regards the prevention of the entry into the legal supply chain of falsified medicinal products

[8] EMA - Recommendation CM/Rex(2024)3 of the Committee of Ministers to member States on reporting of disappearances of medicinal products for human and veterinary use from legal supply chain, EMA/369583/2023 Rev. 19.1

Superseded

[9] REGULATION (EU) 2017/746 OF THE EUROPEAN PARLIAMENT AND OF THE COUNCIL of 5 April 2017 on in vitro diagnostic medical devices and repealing Directive 98/79/EC and Commission Decision 2010/227/EU

[10] 21 CFR Part 4 Subpart B - Postmarketing Safety Reporting for Combination Products

[11[MDCG 2024-4 Safety reporting in performance studies of in vitro diagnostic medical devices under Regulation (EU) 2017/746, April 2024

8 History of changes

Version	Significant changes with respect to previous version	Effective date
CMS D09-50-01-2	• Complete revision of the document due to the integration of the complaint handling processes of Schering and BHC • Additional complaint categories; use of roman ciphers • More general description of the responsibilities of the Quality Organization • General description of the handling of suspected counterfeits • Evaluation of PTCs with AE by Pharmacovigilance • Requirements for investigation of complaints added • Use of the complaint database for documentation required • Reporting requirements for confirmed counterfeits added according to the GSOP on Quality related Issue Management • Examples for complaint categories moved to annex 1 • Update of flow charts according to complaint handling process • Report form for PTCs and suspected counterfeits merged	2008-03-01
115-1	New version established to - transfer of document into new CMS format - assign a new document number according to the new scheme (=simple consecutive numbers) - change the Area of Applicability section to reflect the denomination of BHC Divisions. No change of contents.	2009-04-21
115-2	Complete revision of the document - name change of divisional entities - LDSM (Local Drug Safety Manager) changes to PVCH (Pharmacovigilance Country Head) - New schedules for workflows - New LODE process - Additional Flow Chart LODE - Alignment of GSOP with Global Pharmacovigilance	2012-08-01
115-3	---	Withdrawal before effective date
115-4	Revision of the document (chapter 5.4.3) with regard to FDA Guidance for Industry: Drug Supply Chain Security Act Implementation: Identification of Suspect Product and Notification	2014-11-15
115-5	Harmonization with AH and MC Update with regard to Medical Device project Update with regard to recently published regulatory requirements (e.g. chapter 8 EU GMP Guide) Integration of Dev@Com user letter	2015-09-22

Superseded

	Bundling with GOI 307 and Suppl. 308 and addition of pick list (Annex 8)	
115-6	Implementation of CAPAs of NSAI inspection Bayer Pharma AG (Dec 2015) - Amendment of definitions risk categories III – V - Deletion of the term “Non-incident” - Harmonization of definitions with R&D-SOPs (RD-SOP-0062 and RD-SOP-022)	2016-04-15
115-7	- Editorial changes (wording adapted to New Bayer) - Clarifications of handling of retain and complaint samples including introduction of Decision tree for handling of complaint samples. - Update of Suspected Counterfeit initial complaint categorization: no longer generally initially classified as risk category II - Update of LODE handling according to revised PV process - 3 Retrieval attempts for complaint samples not necessary if justification is provided	2018-02-15
115-8	- Introduction of exemptions from full complaint investigation o Bundling of cases o First Batch Investigation o Abbreviated workflow - Alignment of wording of Responsibilities of all company employees with CP2095 - Harmonization of complaint process also for Radiology - Harmonization of LODE and SLEE handling - Amendment of Annex 7 Pick list for Reporting Suspected Counterfeit Incidents with the column “other Falsified products”	2018-06-15
115-9	Revision of content according to Global Actions from LEV QIP: - Update of FAR in chapter 5.1 and chapter 5.7: A FAR is referenced in the complaint report and in DEV@COM and submitted within 3 working days even if the complaint investigation has not been finalized. - Update of chapter 5.5.1: integration of additional guidance with regard to Batch Record Review.	2019-01-15
115-10	This SOP was transferred from a text-based style to the new Process oriented style to improve the information structure The applied changes are: - Structural changes to follow the process flow, no Quality Standards were changed o Incorporation of contents from GSOP 119 “Handling of Product Technical Complaints received internally and from Third Parties” into this document - Content changes: o Extension of bundling also to serialization alerts	2019-08-15

Superseded

115-11	- Introduction of the role and responsibilities of Shared Service Center - Inclusion of Medical Device and Combination Product defect coding - Removal of first batch investigation (functionality to duplicate low risk complaint cases) - Amendment of Annex 4 Pick lists with regard to EU FMD	2020-05-15
115-12	Expedited revision - Amendment of Chapter 5.2 “Investigate case”: o Chapter 5.2.1: Reference to an investigation already performed if the complaint reason is well understood o Chapter 5.2.2: Remark section added: Testing is designed to confirm the quality of the product/batch by data (FDA Inspection SC LEV) - Requirements for bundling specified (chapter 5.1.1) - Storage time for complaint samples adjusted from “6 months” to “3 months after completion of the investigation” (see chapter 4.1.3)	2020-11-15
115-13	- Animal Health deleted from scope - Update to Annex 4 - Inclusion of content from SUP 1635 “Handling of Product Technical Complaints samples” – Suppl 1635 set obsolete - Reference to Supplement 1636 “RQU Investigation Form for reportable Medical Device Incidents” added. - Introduction of bundling for social media complaints - Removal of requirement to send PTC samples, when other media such as photos are sufficient - Extension of data entry timeline, if neither sample nor batch is available for RC III & IV - Guidance for internal complaints if countries/sites are performing repackaging activities or visual inspections	2022-09-01
115-14	- Process adaption due to migration from Dev@Com into IQMS - Chapter 4.1.1 and Annexes Complaint classification revised - Timelines (chapter 4.1.2) adapted - Initial risk classification task shifted from GBS/CQU to RQU - Wording refined regarding Falsification and counterfeit incidents including adaption of definitions - Shared Service Center changed into GBS - Annex 4 Picklist completely revised and simplified - Supplements 1636 and 1582 set obsolete since reporting is conducted by PV	See coversheet

Superseded

9 Annex 1: Examples for Risk Classification of PTCs

(Most examples taken from "EMA – Compilation of Union Procedures on inspections and Exchange of Information EMA/369583/2023 Rev 19.1)

Risk Classification "High": Complaints regarding defects which are potentially life-threatening or could cause a serious risk to health.

Examples:

- wrong product (label and contents are different products)
- correct product but wrong strength, with serious medical consequences
- microbial contamination of sterile injectable or ophthalmic products or microbial contamination of any medicinal product which is administered to, or taken by, immuno-compromised patients
- chemical contamination with serious medical consequences
- mix up of products within a pack, e.g., two different blister strips within one outer carton, or, two different tablets within the one blister strip.
- Wrong active substance in a multi-component product with serious medical consequences.
- Presence of particle in injectable medicinal products
- The defect presents a high environment risk
- The defect presents a high risk to those who may administer the product to patients

Risk Classification "Medium": Complaints regarding defects which could cause illness or mistreatment with potentially non-serious medical consequences but are not classified as critical.

Examples:

- mislabeling (e.g. wrong or missing text or figures)
- missing or incorrect information relating to labels, leaflets or pack inserts
- Microbial contamination of products that are intended to be non-sterile, with potentially non-serious medical consequence
- Chemical/physical contamination (significant impurities, cross contamination, particles)
- Mix-up of products, e.g., a case of product A contains one or more packs of product B, but A & B are very similar products (e.g. generic versions of a product) and the mix-up does not pose a clinical risk.
- non-compliance with specification (e.g. assay, stability, fill/weight), with risk of lack of efficacy or toxicity. Note: certain lack of efficacy and toxicity issues might be considered to be high risk
- Unsecured closure with non-serious medical consequences

Risk Classification "Low": Complaints regarding defects which may not pose a significant hazard to health.

Examples:

- Unclear labelling, minor labelling errors, e.g. wrong or missing variable data (batch number or expiry date)
- Over-labelling of expiry dates or other information that is executed incorrectly.
- Faulty closures, where no increased risk to the quality of the product is presented
- Contamination, e.g., microbial spoilage, dirt or detritus, particulate matter
- broken immediate container identified before application
- Under-filled or over-filled containers/packs which do not pose a clinical risk.
- single case (i.e. one object, package, etc.) of mislabeling
- occasional empty blister,
- occasional lack of printed material,
- damaged or contaminated secondary packaging,

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- insignificant printing error,
- insignificant occasional device defect
- diverted genuine or fraudulent products without potentially significant health risk

Risk Classification "Non-justified": Non-defective product. Non-justified quality defects are defect reports which could not be substantiated, and which were not true quality defects when they were investigated.

This category is only used for the final classification.

Examples:

- complaints about contamination with particles, where no particles could be found in the sample, suspected counterfeits, which turn out as original product.
- Reports of crystallization in a product where crystallization is a known phenomenon with that product and where the product information (e.g. package leaflet, etc.) provides information on how to deal with that.
- Complaints that relate to the misuse of the product

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10 Annex 2: Report Form for PTC or suspected Falsification and Counterfeit Products

The form can be modified according to specific requirements regarding complaints related to e.g. investigational medicinal products, APIs, intermediates, etc.

To open the Form, please use the following instructions in LifeDoc:

1. Start LifeDoc and Sign-In
2. Select the effective version of GSOP 115 “Manage Product Technical Complaint and Suspected Falsification and Counterfeit Incident”
3. Open the native content from LifeDoc
4. Open the embedded Word file
5. Complete required data



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11 Annex 3: Falsification and Suspected Counterfeit Evaluation Form (SCEF)

Download the following word file.

To open the template, please use the following instructions in LifeDoc:

1. Start LifeDoc and Sign-In
2. Select the effective version of GSOP 115
3. Open the native content from LifeDoc
4. Open the embedded Wordfile
5. Complete required data in the template



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12 Annex 4: Pick list for Reporting Suspected Falsification and Counterfeit Incidents

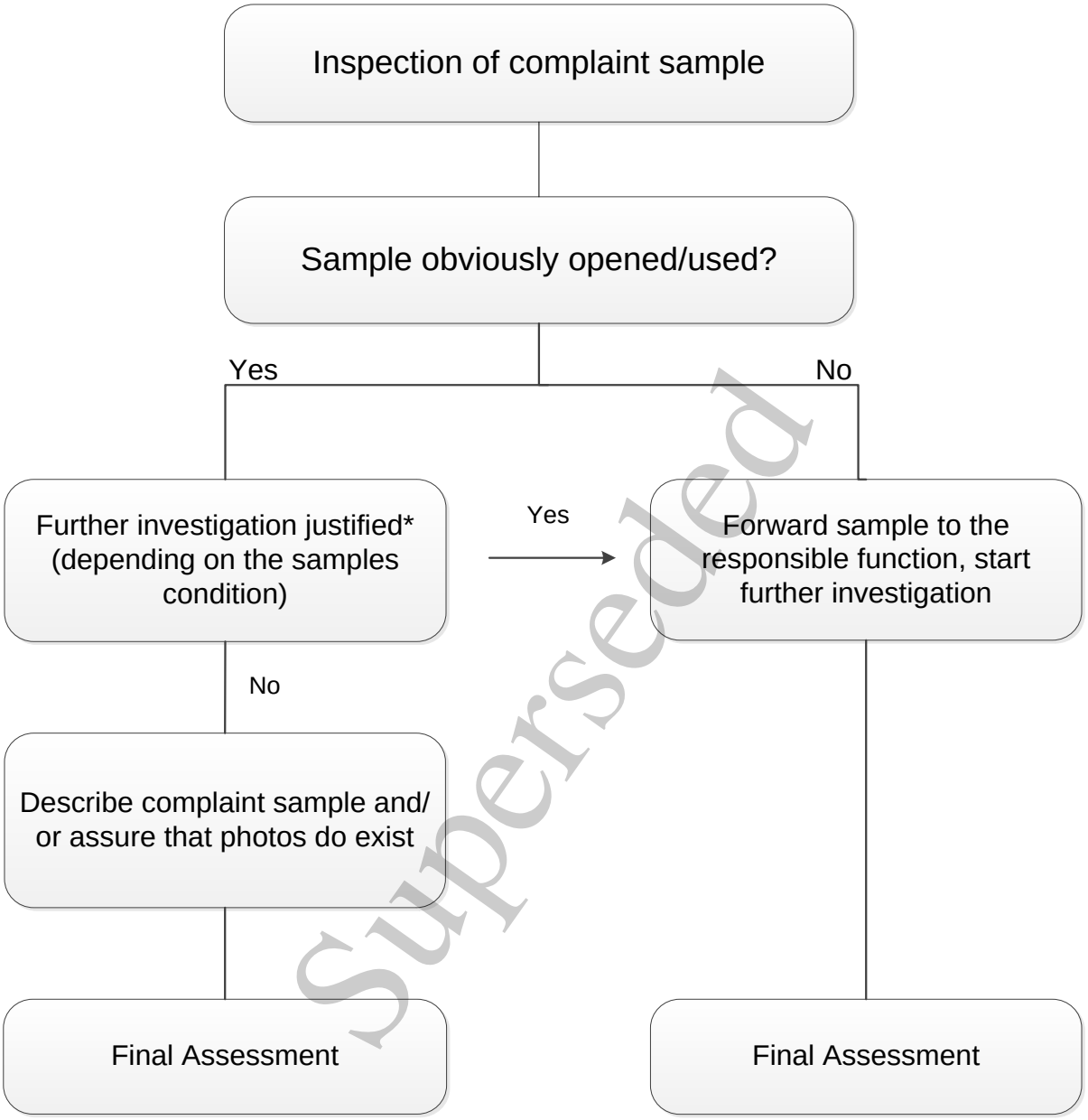
	Falsification (Falsified Medicine and Other Falsified Products (cosmetic, food supplements, non-regulated product))			Counterfeit
Incident subtypes	**(1) Any medicinal product with a false representation** (e.g., of its identity, including its packaging and labelling, its name or its composition (other than any unintentional quality defect) as regards any of its ingredients including excipients and the strength of those ingredients) **(2) Manipulation of genuine products** (e.g., changing variable data, removing content, exchanging content, returning used products)	**Disappeared/Diverted Product (Smuggling)** (Genuine product rerouted out of its intended legal supply chain into another supply chain by unauthorized actions, irrespective of whether the second supply chain is legal or not)	**Disappeared/Theft/Stolen Product** Genuine product taken without authorization from anywhere in the upstream or downstream supply chain (warehouse, transport means, whilst within transit), be it by use of force, threat of force (e.g., robbery), misappropriation (e.g., by personnel) or deception (e.g., fraud)	**(1) Trademark Violation** Product either using without authorization any or a combination of the following: tradename, trademark, icon, rights owned and protected by Bayer according to applicable legislation **(2) Fraudulent Products** Products imitating a "generic" version of a genuine (Bayer) product by using any or a combination of the following (1) visual aspects of the original, (2) naming very close to the original or (3) claiming that the same API is present and independent of its registration status and true composition but excluding generic products with known legal registration status which belong to an identifiable MAH.
Feedback Type	Suspected Counterfeit / Falsification	Suspected Counterfeit / Falsification	Suspected Counterfeit / Falsification	Suspected Counterfeit / Falsification
Title	Name of the reported product: "Suspected falcification" or "Suspected manipulation"	Name of the reported product: "Suspected diversion"	Name of the reported product: "Suspected theft/stolen product"	Name of the reported product: "Trademark violation" or "Fraudulent product"
Counterfeit / Falsification Confirmed?**	Yes	Yes	Yes	Yes*
Conclusion	Confirmed Falsification from Unknown Source Confirmed Falsification in Illegal Supply Chain Confirmed Falsification in Regular Supply Chain	Confirmed Falsification - Diversion from Unknown Source Confirmed Falsification - Diversion in Illegal Supply Chain Confirmed Falsification - Diversion in Regular Supply Chain	Confirmed Falsification - Theft from Unknown Source Confirmed Falsification - Theft in Illegal Supply Chain Confirmed Falsification - Theft in Regular Supply Chain	Confirmed Counterfeit (Trademark Violation) Confirmed Counterfeit (Fraudulent Generic) from Unknown Source Confirmed Counterfeit (Fraudulent Generic) in Illegal Supply Chain Confirmed Counterfeit (Fraudulent Generic) in Regular Supply Chain

Superseded

* For Trademark Violations, where known that the reported incident does not refer to a Bayer product, assessment is performed by GBS/CQU

**Cases reported as "suspected falsification / counterfeit" and assessed to be genuine/registered product follow regular PTC process

13 Annex 5: Decision tree for handling complaint samples



Superseded

* No further investigations (unless otherwise requested by RQU) necessary if:

- part of relevance for investigation is not present in complaint sample
- obvious handling errors are present or frequently observed or
- the batch and the reason for complaint have already been investigated and no further results are expected by investigating of the samples

Document Approval Electronic Signatures

Signed by	Meaning of Signature	Server Date (yyyy-MM-dd HH:mm 'UTC')
Huelsmann, Stefan (sgoge)	Approve - Content Author	2024-10-18 07:56: UTC
Schnittger, Katrin (ggnha)	Approve - Management	2024-10-18 16:23: UTC

Superseded